

PRINTABLE DESK KIT

The Printable PT Desk Kit

15 Quick References · 5 Patient Handouts · 3 Burnout Mini-Tools · 1 Custom Template

Print these pages. Laminate the ones you use daily. Tape them to your desk.
Keep a copy in your bag. These are the sheets you reach for when you need a
script fast.

FROM THE PT COMMUNICATION RESOURCE

The Quiet Shift Series · Sovereign System Productions · 2026

QUICK REFERENCE 1

"When a Patient Says X, Try Y"

PATIENT SAYS	TRY SAYING
"I didn't have time."	"That is real. What is one small thing you could fit in, even on a busy day?"
"It hurts."	"Show me where. Let us find a version that does NOT hurt."
"I will do it later."	"Later rarely comes. What is the smallest thing you could do right now?"
"I don't feel ready."	"You may never feel ready. Can we try one small step and see how it feels?"
"This is not working."	"Tell me more. What were you expecting that is not happening?"
"I am fine."	"Let me show you what 'fine' looks like in a strength test — sometimes we have hidden gaps."
"I already tried that."	"Can you tell me what happened? That way we do not repeat what did not work."
"My friend said..."	"Every body is different. What worked for your friend might not work for you."
"I am just getting old."	"Aging changes things. But it does not sentence you to pain."
"I don't want to be a burden."	"You are not a burden. Asking questions helps me help you."

QUICK REFERENCE 2

Top 15 Phrases Patients Misunderstand

WHAT PTS SAY	WHAT PATIENTS HEAR	BETTER PHRASE
"Take it easy."	"Do nothing."	"Do a little less than you think you can."
"Listen to your body."	"Stop if it hurts at all."	"Pain is information. Tell me what you feel."
"You will get there."	"Not today."	"Here is what 'there' looks like. Here is the bridge."
"Consistency matters."	"Do it perfectly every day."	"Showing up counts even on the bad days."
"Rest is important."	"Rest is lazy."	"Rest is when your body repairs. It is part of the plan."
"Do not overdo it."	"Do not push at all."	"Find the edge. Stop before you fall off it."
"Patience."	"Stop complaining."	"This stage is frustrating. That is normal."
"Everyone is different."	"I have no answer."	"Your body has its own timeline. Here is what we control."
"Trust the process."	"Trust me blindly."	"Here is the evidence. Let us track progress together."
"You are doing great."	"You are done."	"You have come far. Here is what is next."
"Modify as needed."	"Do whatever you want."	"Here is the safe range. Modify within these boundaries."
"Worse before better."	"Expect more pain."	"Temporary soreness is normal if it settles."
"Quality over quantity."	"Do fewer reps."	"Doing these perfectly matters more than doing many poorly."
"Stay positive."	"Ignore your pain."	"Your mindset matters — but so does your pain."
"You have to want it."	"If not better, your fault."	"Effort matters. But your body has its own timeline too."

QUICK REFERENCE 3

First Visit Script (Setting Expectations)

5-Minute Script for the First Visit

"Here is how I work. I am going to teach you things. You are going to practice at home. The people who get the best results are the ones who show up — even on days they do not feel like it.

Recovery is not a straight line. You will have good weeks and hard weeks. That is normal. When a hard week happens, do not quit. Tell me. We adjust.

My job is to give you the right plan. Your job is to do your best with it. If something is not working, that is my fault until we fix it together. Any questions before we start?"

Printable Version for New Patients

Keep this card. Read it again when you are frustrated.

Recovery has ups and downs. Setbacks are not starting over.

Tell me when something is not working. I will adjust.

Showing up on bad days counts more than perfection on good days.

You are not a burden. Asking questions helps me help you.

QUICK REFERENCE 4

"I'm Fine" Visit Script (Early Discharge)

5-Minute Script for the Patient Ready to Discharge Early

"I am glad you are feeling better. That means phase one worked. But there is a difference between feeling better and being stronger. Feeling better can vanish in a week. Being stronger stays with you."

"Here is what I want you to consider: let us finish the full plan. Not because you need to feel better — because you want to stay better. If you still want to discharge today, I will support that. But I want you to know what you are leaving behind."

If They Discharged Early and Relapsed:

"This happened because we stopped too soon. That is not your fault. I should have been clearer. Now we know. Let us finish the full plan this time. No shortcuts."

QUICK REFERENCE 5

Frustrated Patient Script (Plateau/Setback)

5-Minute Script for the Patient Who Has Plateaued

"I hear your frustration. You were moving forward, and now you feel stuck. That is hard."

"Here is what I know: plateaus are normal. They are not a sign of failure. They are a sign that your body needs something different. So let us do something different. I am going to adjust the plan. Not because you did anything wrong — because that is my job."

"Can we agree to try this new plan for two weeks before we decide if it is working?"

For the Patient Who Wants to Quit:

"I understand why you want to quit. But let me ask you: if you quit now, what happens in six months? Does this problem go away, or does it come back worse? Let us try one more adjustment before we give up."

QUICK REFERENCE 6

Non-Adherent Patient Script (Gentle)

5-Minute Script for the Patient Who Is Not Following Through

"I have noticed you have been having a hard time keeping up with the home exercises. That happens. It is not a moral failure."

"Let me ask you: is the problem motivation, time, pain, or something else? Be honest. I have heard it all."

"Once I know the real barrier, we can solve it together. Smaller plan. Different exercises. A reminder system. Whatever you need. The only wrong answer is pretending everything is fine when it is not."

If They Admit They Haven't Been Trying:

"Thank you for being honest. That is brave. Now let us fix it — together. What is the smallest, easiest version of this plan that you could actually do?"

QUICK REFERENCE 7

Red Flags vs. Yellow Flags

RED FLAG (STOP. REFER IMMEDIATELY.)	YELLOW FLAG (PROCEED WITH CAUTION.)
Sudden, severe, unexplained pain	Gradual worsening over weeks
Loss of bladder or bowel control	Mild morning stiffness
Progressive weakness in limbs	Fatigue that improves with rest
Chest pain or shortness of breath	History of similar episodes
Fever with unexplained MSK pain	Stress or mood affecting symptoms
Night pain that wakes from sleep	Mild swelling without redness or heat
Unexplained weight loss	Deconditioning from inactivity
Trauma with possible fracture	Patient frustration or low motivation

Red Flag Script:

"I am concerned. This is outside what I can safely treat. You need to see your doctor — today if possible. I will write down what I observed for you to take with you."

Yellow Flag Script:

"This is not an emergency, but we need to be careful. Let us start very conservatively, and I want you to tell me immediately if anything changes."

QUICK REFERENCE 8

The 5-Minute Morning Check-In for PTs

For you, not for patients. Do this before your first patient.

1 min — Body scan: Am I carrying tension? Did I sleep enough? Am I hungry? Fix it now.

1 min — Emotional check: How am I feeling? Am I carrying anything from yesterday? Name it. Set it down.

1 min — Intentions: What is one thing I want to do well today? Who is the one patient I need to be most present for?

1 min — Boundaries: What is one request I will say no to? What is one thing I will protect?

1 min — Quiet shift: Three slow breaths. "I am here. I am enough. I will help who I can."

QUICK REFERENCE 9

Patient Personality Types

PERSONALITY	WHAT THEY NEED	WHAT TO SAY	WHAT TO AVOID
The Skeptic	Evidence, logic, proof	"Here is what research shows..."	"Trust me" or emotional appeals
The Pleaser	Permission to be honest	"I need your real answer, not the nice answer."	Assuming they are fine
The Overwhelmed	Simplicity, small steps	"Let us focus on just one thing this week."	Information overload
The Competitive	Metrics, comparison to self	"Last week you did X. This week you did Y."	Comparing to others
The Victim	Empowerment, choice	"You are in control. Here are your options."	Sympathy (traps them)
The Impatient	Honest timelines, fast wins	"Here is what you can expect week by week."	Vague answers
The Denier	Gentle reality, no shame	"Let us look at the data together."	"You are wrong"
The Dependent	Boundaries, self-efficacy	"I will teach you. You will do it."	Doing things for them

The 10 Most Common Patient Fears

FEAR	WHAT TO SAY
"I will make it worse."	"We will start so small that making it worse is almost impossible. You are in control."
"I am too old."	"Age is not a disease. I have helped people older than you get stronger."
"I have tried everything."	"Tell me what you tried. Let us figure out what was missing."
"I do not have time."	"Let us find a version that takes five minutes or less."
"It will never get better."	"I cannot promise full recovery. But I can promise you can get better than today."
"I am afraid of surgery."	"Let us do everything possible to avoid it. Surgery will still be there if you need it."
"I am letting my family down."	"Taking care of yourself is how you take care of them."
"I do not trust my body."	"Trust will come back — one small success at a time."
"I am in too much pain."	"Then we start with what does NOT hurt. There is always something."
"What if nothing works?"	"We will not know until we try. Let us try. Then we will know."

HEP Prescription Cheat Sheet

Starting Point (Most Patients)

VARIABLE	RECOMMENDATION
Number of exercises	2-4
Frequency	3-5 days per week
Sets	1-2
Reps	8-12
Hold time (static)	10-30 seconds
Rest between sets	30-60 seconds

When to Progress

Patient completes prescribed frequency for 2 weeks consistently • Pain <3/10 and settles quickly • Exercises feel "too easy"

How to Progress (Choose ONE at a time)

Add reps (12 → 15)	Add sets (2 → 3)
Add resistance (lighter → heavier band)	Add hold time (10s → 20s)
Add complexity (two planes of motion)	Add frequency (4 days → 5 days)

When to Regress

Pain >4/10 that persists after session • Consistently misses sessions • Poor form despite cueing

Moelands et al. (2025): Higher dose = lower adherence. Sometimes less is more.

Motivational Interviewing Phrases for PTs

PRINCIPLE	PHRASE
Ask permission	"Would it be okay if we talk about your home exercises?"
Evoke their reasons	"What concerns do you have about your current activity level?"
Reflect without judgment	"It sounds like you are feeling overwhelmed. That makes sense."
Ask about the gap	"On a scale of 1-10, how important is getting better? Why not lower?"
Ask about confidence	"How confident are you? What would make it higher?"
Elicit change talk	"What would be different if you could move without pain?"
Roll with resistance	"You do not have to do anything you do not want to. I am just giving you information."
Summarize	"You want to get better. Not sure you can fit it in. Did I get that right?"
Ask for commitment	"What are you willing to try between now and next week?"

QUICK REFERENCE 13

What to Say When YOU Are Having a Bad Day

Say these to yourself. Not to patients. Keep this page where you will see it.

Morning scripts:

"I am not a machine. Today is about presence, not performance. Show up. That is enough."

"This patient is not giving me a hard time. They are having a hard time."

Mid-day scripts (after a hard session):

"That was not my best work. That is okay. I will try again next time."

"I did not cause their problem. I cannot control their choices. I can only offer my best."

End-of-day scripts:

"I will document efficiently, then leave work at work."

"Today was hard. I showed up anyway. That is not nothing. That is everything."

QUICK REFERENCE 14

"Things I Wish My Patients Knew"

Cut out and keep at your desk. Or adapt for a patient handout.

If my patients understood just one thing, it would be this:

Recovery is not a straight line. Setbacks are not starting over.

Doing a few exercises well beats doing many exercises poorly.

Your body adapts slowly. Trust the process, not your impatience.

Pain is information, not always damage.

Rest is not weakness. It is when your body repairs.

I am on your side. Tell me when something is not working.

Showing up on bad days counts more than perfection on good days.

You are not a burden. Asking questions helps me help you.

Your effort matters — but your body also has its own timeline.

I cannot promise you full recovery. But I can promise you can get better than you are today.

When You Need to Refer Out: Decision Guide

For those "is this mine to treat?" moments

Red Flags — Refer to Physician Now

Sudden unexplained weight loss • Night pain not responding to position change • Bowel/bladder dysfunction (cauda equina) • Progressive neurological deficits • Severe trauma with suspected fracture • Suspected DVT • Chest pain or shortness of breath

Yellow Flags — Consider Mental Health Referral

Persistent fear-avoidance beyond 4-6 weeks • Catastrophizing language • Signs of depression • History of trauma surfacing in sessions • Suicidal ideation — refer immediately

Scope Boundary Moments — Refer to Specialist

Complex vestibular/neurological beyond training • Pediatric developmental concerns • Wound care beyond superficial integrity • Suspected systemic disease • Persistent symptoms unresponsive to 4-6 visits

"This Is Beyond My Lane" Self-Check

Am I treating this because I can help — or because I do not want to lose the patient? Am I qualified? Would I want another PT treating this if it were my family member?

Referral Script: *"I want to make sure you get the best care. This is outside what I treat most effectively. I am going to connect you with [specialist] — and I will coordinate with them so nothing falls through the cracks."*

5 Printable Pages for Your Patients

Print and share. Adapt the language. These are starting points.

1. Why Strengthening Takes Time

Your body heals and strengthens gradually — like a garden, not a light switch. Consistent small actions over time produce lasting change. Do not expect overnight results. Expect steady progress.

2. Why 2-4 Exercises Is Often Enough

Fewer exercises, done regularly, beat many exercises done occasionally. Focus on quality and consistency — not quantity. Two to four movements, done well, is often the sweet spot.

3. Why Pain Doesn't Always Mean Damage

Pain is not always a sign of harm. Sometimes your nervous system is just on high alert. Gentle, safe movement often helps turn down the volume.

4. Why Rest Days Matter

Rest is when your body repairs and strengthens. You are not being lazy. You are following the plan. Elite athletes take rest days. So should you.

5. Why Movement Helps Recovery

Too much rest can weaken muscles and stiffen joints. Gentle, regular movement helps recovery. Find the "just right" amount.

BURNOUT MINI-KIT

Tool 1: The 15-Minute Documentation Reset

For the days when documentation is drowning you.

Step 1 (2 minutes): Close all unnecessary tabs. Phone face down. "I am documenting for 15 minutes. That is all."

Step 2 (10 minutes): Use templates. Bullet points. Only what is necessary for billing and legal.

Step 3 (3 minutes): Stop. Even if not done. Set timer for next block. Walk away. Breathe.

Template for rushed notes:

*"Patient reports [progress/barrier]. HEP reviewed/adjusted. Patient demonstrates [ability/deficit].
Plan: [next step]. Return [date/time]."*

Tool 2: The "Good Enough" Permission Slip

For the perfectionist who cannot stop editing notes at 9 PM.

"My notes do not need to win a Pulitzer. They need to be accurate and done. Good enough notes that are done are better than perfect notes that are late."

Now: Save the note. Close the chart. Walk away from the computer. You are done for today.

BURNOUT MINI-KIT

Tool 3: The Post-Shift Wind-Down

For when you carry the day home with you.

First 5 minutes: Put your bag down. Do not check email. Change clothes. Drink water. (You forgot. You always forget.)

Next 10 minutes: If a patient interaction is stuck in your head, write ONE sentence about it. Then close the notebook. Say out loud: "I did my best today. That is enough. The rest is not mine to carry."

Last 5 minutes: Name one thing that went well today. Name one thing you will do differently tomorrow. Then stop thinking about work.

CUSTOM TEMPLATE

My Most-Used Scripts

Write down the scripts that work for you. Keep them here. Update them when you find something better.

SITUATION	MY SCRIPT
Patient hasn't done HEP	
Patient is afraid of pain	
Patient wants more exercises	
Patient is frustrated	
Patient cancels repeatedly	
Patient asks about surgery	
Other:	
Other:	
Other:	
Other:	

THIS IS JUST THE DESK KIT

**The Full Book Has 35 Scripts + 25 Explanations + 30 Q&As +
Full Burnout Kit + AI App**

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